



Sexual Safety: Responding to Incidents Policy

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPG)	All inpatient, Community Mental Health Services (CMHS), Outpatient areas and Safe Haven	All Clinical and Administrative* Staff

Policy statement

- RPG has a duty of care and legal obligation to provide health care in an environment that promotes personal safety and minimises risk of inappropriate behaviour, including that of a sexual nature, allegedly perpetrated by patients, visitors, staff, students, contractors, and volunteers.
- This policy will guide staff responses to allegations of or suspected sexual safety incidents, whether historical or recent, within RPG services.

* Note: Administrative staff receiving disclosures of sexual safety incidents are required to sensitively respond to the patient and immediately report to an available clinician - they are not responsible for patient assessment or clinical care provision.

For the purpose of this document patients and consumers across all settings will be referred to as patients.

- Refer to the [Sexual Assault Resource Centre \(SARC\)](#) website for information and the following documents where necessary:
 - [RPG ED Presentation of Alleged Sexual Assault Management SOP](#)
 - [RPG Sexual Safety Policy](#)
 - [RPG Recognising and Responding to Family and Domestic Violence and Elder Abuse SOP](#)
 - [East Metropolitan Health Service \(EMHS\) Sexual Safety in Mental Health Policy](#)
 - [WA Department of Health Procedure for Responding to a Recent Sexual Assault](#)
 - [WA Department of Health Responding to an Allegation of Sexual Assault Disclosed within a Public Mental Health Service Policy](#)
 - [WA Department of Health Responding to the Abuse of Older People \(Elder Abuse\) Policy](#)
 - [WA Department of Health Safety Planning for Mental Health Consumers Policy](#)
 - [Child and Adolescent Health Services \(CAHS\) Guidelines for Protecting Children \(2020\)](#)

Staff are expected to already be familiar with the **RPG Sexual Safety Policy**.

Contents

Definitions	3
Policy directives	4
Indicators	4
Response process	4
1. Listen to and acknowledge disclosure, invite patient to a more appropriate setting to discuss - or escalate to senior staff	4
2. Ensure immediate safety and consider the need for assistance including:	5
3. Provide physical examination and care - medical care is a priority	5
4. Establish a brief history of what has happened and when to determine an appropriate course of action.	6
5. Provide psychological support.....	6
6. Manage capacity and consent issues.....	7
7. Escalating sexual safety incidents to senior staff – including retracted disclosures	9
8. Plan for ongoing care and discharge.....	9
9. Risk assessment and management of alleged perpetrator	10
10. Notifications, mandatory reporting, and documentation	11
11. Debrief.....	12
12. Investigation leads.....	12
Other scenarios to consider	13
Concerns about the plausibility of a patient’s mental state.....	13
Retracted or suspected disclosures	13
Where a sexual safety incident is reported by a third party	13
Allegations against a RPBG staff member	13
Compliance monitoring	13
Appendix I: Responding to a Sexual Safety Incident: Overview	17

Definitions

EEK (Early Evidence Kit)	Forensic specimens may be self-collected by the individual using the EEK in accordance with the instructions enclosed in the kits. EEK supplies can be ordered via the Health Corporate Network.
SARC Forensic Kits	These kits (female and male) contain all the documents and specimen containers required for a sexual assault forensic examination by a medical doctor and meet the National Forensic Standards for Minimising DNA contamination and are admissible as evidence in court.
Sexual safety incident	A range of sexual acts and behaviours committed against a person without their consent, including • Sexual assault (a crime of violence) – e.g., any unwanted sexual act or behaviour which is threatening, violent, forced, or coercive and to which a person has not given consent or was not able to give consent: ¹ (e.g., unconscious, asleep, intoxicated, having a developmental disability or mental health issues significantly impairing decision making) • Harassment – e.g., unwanted, or unwelcome sexual behaviour where a reasonable person would have anticipated the possibility that the person harassed would feel offended, humiliated, or intimidated ² • Being spoken to using sexualised language • Sexually disinhibited behaviour and/or observing other people behaving in a disinhibited manner e.g., nakedness and exposure or masturbation, being made to watch or shown pornographic images.³ Also, consensual sexual activity within an inappropriate context or setting e.g., any public or private areas within health setting.
<u>Sexual Assault Resource Centre (SARC)</u>	A free, 24/7 service related to sexual safety incidents and sexual assault for persons aged 13 years and over. Access further service information SARC. SARC doctors and counsellors work from premises on the King Edward Memorial Hospital (KEMH) as an out-patient clinic. • If the patient is medically safe, they can be seen at SARC regardless of whether they intend to report to the police • If the patient requires continuing Emergency Department (ED) or hospital treatment, the SARC doctor and counsellor can come to the hospital or service • Some patients may benefit from having the service of SARC explained directly to them by a SARC counsellor.

Policy directives

Staff are to ensure timely and proportionate response to all disclosures and reports of sexual safety incidents, whether historical or recent, as per the principles of care outlined in the **R PBG Sexual Safety Policy**, **EMHS Sexual Safety in Mental Health Policy** and [WA Department of Health Responding to an Allegation of Sexual Assault Disclosed within a Public Mental Health Service Policy](#).

- Patient consent must be obtained before forensic examination can be conducted and all medical doctors can conduct forensic examination and do not require SARC training
 - Only appropriately trained nurses i.e., nurses who have completed SARC training (predominantly available to country services) and received a certificate from the police can conduct forensic examination
- Forensic sampling is to be conducted by a SARC health professional or may be performed in ED or, during rare circumstances of unexpected delays or availability, by an R PBG medical doctor under the direction of or advice from the SARC doctor
- When a doctor is not available to conduct a full forensic examination, an EEK must be offered to the individual for specimen collection (See [WA Department of Health Procedure for Responding to a Recent Sexual Assault](#))

Indicators

Indicators that a sexual safety incident may have occurred whether historical or recent include:

- Direct disclosure by the person
- Direct observation by a staff member and/or other witness
- Suspected based on physical symptoms or a change in behaviours.

Response process

Clinician's response to disclosure of or following observation of an alleged sexual safety incident:

1. Listen to and acknowledge disclosure, invite patient to a more appropriate setting to discuss - or escalate to senior staff

- If the report is made via telephone (e.g., community patient), arrange for a clinician to meet with the patient face to face to discuss
- Check communication skills and interpreter needs - ensure a professional interpreter is present if required. Family, friends, or other non-qualified interpreters are not to be used
- Consider patient wishes regarding support people present, e.g., next of kin (NoK), carer, significant other or personal support person (PSP)
 - Family member may be appropriate unless determined by a senior clinician (e.g., Consultant) to not be in the best interest of the patient, in which case a more appropriate alternative support is to be considered
- Promote dignity by ensuring access to appropriate clothing where necessary.
- Consider capacity, consent, and any limits of confidentiality (e.g., mandatory reporting).

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Version: 3.0
 Endorsed by: Clinical Practice Committee 07/12/2023
 Authorised by: No Patient Harm Committee 28/05/2024

Date Compiled: 03/2018
 Document owner: Director of Clinical Services
 Revision Due: 05/2027

Response process cont'd

2. Ensure immediate safety and consider the need for assistance including:

- Code Black
- Code Blue
- 000 or assist patient to the nearest ED if not at Royal Perth Hospital (RPH)
- Consider the safety and wellbeing of other patients and staff, escalate to senior staff immediately and act accordingly – e.g.
 - Employ additional staff, increased observations, 1:1 nurse specials, or transfer patients where applicable. Refer to [Special and Supportive Observations Policy](#)
- Consider actions if patient discharges against medical advice (DAMAs), initiates own discharges, or leaves without notice (refer to [EMHS Discharge Against Medical Advice \(DAMA\) Policy](#) and [EMHS Management and Review of 'Did Not Wait' Patients that Present to Emergency Departments Policy](#)).

3. Provide physical examination and care - medical care is a priority

- Follow the ABCDE protocol (Airway, Breathing, Circulation, Disability, Exposure), including consideration of the following:
 - Provision of urgent, lifesaving treatment of any injuries and pain management is more important than forensic collection
 - consult with SARC team prior to initiating pain management to minimise impact on capacity to consent, plan forensic examination and/or other individual care needs
 - Patients may present directly to the ED
 - Emergency contraception and/or possible current pregnancy
 - Screening for sexually transmitted infections (STI) and blood borne viruses (BBV). Refer to [ED Post-Exposure Prophylaxis Non-Occupational Exposure to Blood and Body Fluids Clinical Guideline \(RPH\)](#).
- For patients in the community:
 - Provide lifesaving treatment where required
 - Go directly to the ED
 - Refer to the [ED Presentation of Alleged Sexual Assault Management SOP](#)
 - or
 - Encourage patient to go directly to ED or present to a general practitioner (GP)

For all patients contact SARC Duty Doctor for management advice and referral where patient consents:

Mon-Fri 09:00-16:00 hours 08 6458 1820

After Hours via KEMH switchboard 08 6458 2222

Note: SARC doctors can only examine and collect forensic evidence from a patient who has capacity and can give informed consent

Response process cont'd

4. Establish a brief history of what has happened and when to determine an appropriate course of action.

Clinicians must use [SARC Emergency Care: History and Checklist \(Metro\) form](#) in conjunction with the information below.

- Mandatory review requirements
 - Age:
 - Under 13 years contact Child Protection Unit (CPU) 08 6456 4300
 - If 13-15 years contact either SARC or CPU
 - 18 years and over - consider contacting SARC on 1800 199 888 or via KEMH switchboard 08 6458 2222

Comply with mandatory reporting requirements for children and young people under 18 years via [Mandatory Reporting Information System \(communities.wa.gov.au\)](#). Refer to [CAHS Guidelines for Protecting Children](#)

- Capacity
 - If diminished, escalate to consultant
 - Consider liaising with Social Work regarding reporting to the Police and respond appropriately
 - Consider notifying parent, guardian, NoK or significant other if applicable and appropriate. Refer to [WA Office of the Public Advocate - Making Decisions: OPA information](#)
- Communication skills and/or interpreter needs
- Non-urgent medical issues to include emergency contraception, testing for sexually transmitted infections (STIs), minor injuries, pre-existing conditions, and pregnancy in consultation with SARC
- Any history of non-fatal attempted strangulation, previous trauma and/or assault
- Involuntary status under the **Mental Health Act 2014 (MHA)**

5. Provide psychological support

- Offer reassurance and sensitivity
- Complete or update relevant risk assessment regarding the following concerns:
 - Mental health (MH)
 - If the patient is assessed as at risk of serious self-harm they may require involuntary admission to hospital under **MHA**. Escalate accordingly – e.g., call relevant emergency services, bring patient onto RPBG site, or follow protocols as per [Recognising and Responding to Acute Deterioration \(RRAD\) Mental State, Behaviour, Cognition SOP](#) and [EMHS Sexual Safety in Mental Health Policy](#)
 - Physical health – refer to [RRAD Policy](#)
 - Personal safety
 - Family and domestic violence
 - Child abuse
 - Elder abuse

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Response process – 5. Provide psychological support cont'd

- Support the patient to be protected from further contact with the alleged perpetrator when in the care environment, regardless of whether this is a staff member, patient, family member, carer, PSP, or visitor
- If there is no immediate danger, the patient is to be asked if they've previously disclosed the assault or abuse and received any support or treatment
- Provide information (written and verbal) regarding available help (e.g. SARC's [Coping after a sexual assault](#) brochure) and treatment options (including external support e.g., SARC, refuge and emergency accommodation, domestic violence services, legal services, Centrelink, Crisis Care), processes, rights, and limits of confidentiality
 - Refer to Social Work Department, Social Work Outpatients and/or Allied Health as applicable
 - Offer choices where possible
 - Consider patient's gender and gender identity
 - Refer to [Recognising and Responding to Family and Domestic Violence and Elder Abuse SOP](#) and [WA Department of Health Responding to the Abuse of Older People \(Elder Abuse\) Policy](#).

6. Manage capacity and consent issues

Refer to [WA Department of Health Consent to Treatment Policy](#) and [Triage to Discharge MH Framework for SSCD](#).

- Where patient has capacity seek consent regarding:
 - Contacting NoK, PSPs and significant others
 - Referral to SARC who are gazetted health professionals, to conduct the examinations listed below and/or RPH ED/RPBG medical doctor:
 - Physical examination (where not life threatening) and collection of medical specimens
 - Genital examination
 - Forensic examination and collection of forensic specimens **Note this requires written consent**
 - Ideally, medical, and forensic examination and treatment should be completed simultaneously, with patient consent
 - Forensic sampling is time sensitive and is to be preferentially conducted by a SARC health professional or a RPBG medical doctor in liaison with the SARC doctor at the **first possible opportunity** or within 12 hours from initial presentation and gathering of evidence (but must not compromise urgent medical care). In some cases, forensic evidence may only be available for up to 24 hours after the assault. After 10 days, it is highly unlikely that forensic evidence can be collected. Refer to [SARC website](#)
 - Medical Practitioner to discuss medical history and physical assessment with SARC health professional
 - Encourage the patient not to shower, wash or pass urine prior to examination
 - Consider that forensic examination may be performed in ED to allow the patient to eat, drink and void, without loss of evidence

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Response process – 6. Manage capacity and consent issues cont'd

- A Chaperone **should be present at ALL times** during the interview and any physical examination (refer to [EMHS Chaperone Policy](#))
- Obtain consent for each component of treatment, examinations, and evidence collection before proceeding
- Ensure use of **SARC Forensic Kit**
- If applicable, SARC health professional or RPBG doctor to package and store forensic specimens to maintain chain of custody as per the **SARC Medical and Forensic Manual**
- Call SARC for advice, if required.
- If delay in assessment, offer an EEK to the person for specimen collection.
- Reporting to the police (Sex Assault Squad - WA Police 08 9428 1600) - encourage this
 - Any report to the WA Police by a patient over 18 years must be the patient's decision. **Staff may assist sharing information with WA police with patient's written consent**
- Remaining in hospital if suffered injuries - where in best interests of patient
- Where capacity is uncertain
 - Escalate to Consultant Psychiatrist or On call Consultant for an MH assessment prior to forensic examination
 - Mental State Examination (MSE) to be undertaken as appropriate
 - **Note:** An involuntary patient under the MHA may have capacity
 - Where patient is assessed as unable to make an informed decision refer to WA Office of the Public Advocate's Hierarchy of Treatment Decision Makers
 - Liaise with appointed guardian as applicable
 - Discuss with patient to determine whether they agree to their NoK or significant other being informed
 - Consult with the WA Police Child Abuse Unit when the patient is under 18 years of age, to determine the required action to be taken
- Senior clinicians are to seek advice regarding reporting to Police in the public interest and/or the need for forensic examination.

Note: reporting to police without patient consent requires Tier 2A officer approval under [EMHS Delegations and Authorisations Schedule](#).

Response process cont'd

7. Escalating sexual safety incidents to senior staff – including retracted disclosures

- Inpatient, Community and Outpatient areas
 - Staff to immediately report details of the incident to the patient's lead clinician, treating team and senior staff:
 - In Hours: Senior manager ward, unit, Community or Outpatient Services
 - After hours: Operations Hub Coordinator (OHC), After Hours (AH) Bentley Health Service (BHS) MH Clinical Nurse Specialist (CNS), Duty Medical Officer (DMO), Senior Nurse on Call (SNoC) or On Call Consultant Psychiatrist
 - Relevant senior clinicians (as above) will coordinate response and determine appropriate strategies to ensure the patient's safety is immediately considered and escalate further as necessary
 - This may include movement of room, ward and/or increased visual observations
 - AH BHS MH CNS decides regarding informing the SNoC
 - Treating Consultant informs HoD Psychiatry (RPH), HoD (Community MH), Co-Directors, or On Call Executives
- Safe Haven
 - Peer Workers will immediately report details of the incident to the Clinician, or CNS. Refer to [Safe Haven: Access and Provision of Care SOP](#)
 - Clinician is to complete a *SARC Triage Form* and raise a medical emergency team (MET) call if concerned about physical health and criteria met, or transfer patient to ED for assessment where necessary.

8. Plan for ongoing care and discharge

- Assess ongoing related medical, MH and psychosocial needs and how best to meet them
 - Consider the need for nurse special, companion, increased frequency of observations, medications, follow up investigations and results. Refer to [Special and Supportive Observations Policy](#)
- Develop safety plans and explore advance statements supporting the patient's sexual safety, to be recorded and accessed in the future should the patient need to access services again – e.g.
 - A *What Matters Most form* or crisis awareness plan may include specifying that the patient will not be provided with the same unit, room, or bed where they were previously assaulted, as relevant
 - An alert might be placed on iClinical Manager (iCM) or Psychiatric Services Online Information System (PSOLIS or WebPSOLIS) to flag high risk relating to sexual safety vulnerability.

Response process – 8. Plan for ongoing care and discharge cont'd

- Discharge and handover information is to include:
 - Clear and factual descriptions of breaches of sexual safety
 - Transfer of care processes, referrals and supports required for the patient and consider support for family and significant others as applicable
 - Post discharge follow up requirements
 - Who – e.g., GP, CMHS
 - What – e.g., investigation results, pregnancy testing, repeat testing for STI and BBV
 - When – as discussed with patient

9. Risk assessment and management of alleged perpetrator

If perpetrator is a patient of the health service

- Staff to immediately report details of the incident to alleged perpetrator's lead clinician, treating team and senior staff – see [7. Escalating sexual safety incidents to senior staff](#) section
 - If applicable, seek input from Consultant-Liaison (C-L) Psychiatry Team, Safety all Hours for Everyone team - SAFE for Royal Peth Hospital (RPH) and BeSAFE for Bentley Health Service (BHS)
- Refer or arrange an assessment of the alleged perpetrator's current physical and emotional well-being and capacity and ensure support by an appropriate clinician
- Provide psychosocial support and offer information regarding rights and process for reporting to Police.
- Do not disclose perpetrator's details to police unless consent is obtained and if consent provided, staff to document in medical record before making police report.
- With consent, SARC health professional or RPBG doctor to store separately in sealed, labelled paper bags all articles of clothing (if accessible) worn by alleged perpetrator (in addition to those worn by the victim) at the time of the incident (if applicable).
- Consider the use of increased observations or a 1:1 nurse special and transferring person to another unit, service or ward and consider staff gender allocation where applicable – ask person for preference where practicable
- Also refer to [Other scenarios to consider](#) section.

If perpetrator is not a patient of the health service

- Report to Clinical Lead
- Document allegation in patient medical record
- Also refer to [Other scenarios to consider](#) section.

Response process cont'd

10. Notifications, mandatory reporting, and documentation

- Reporting within RPBG
 - All sexual safety incidents including retracted disclosures, must be documented and reported to the patient's treating team, lead clinician or Consultant where the following criteria is applicable:
 - a) Mandatory reporting requirements apply
 - b) The incident occurred within the health service setting
 - c) The allegation involves a staff member
 - d) The person disclosing the allegation is assessed as lacking capacity
 - If none of the above criteria is met i.e., a – d, and the person has requested that their disclosure remain confidential and not be disclosed further, then the staff member is to:
 - Consider if there has been a capacity assessment check on the person to ensure they can make such a decision (refer to [6. Manage capacity and consent issues](#) section)
 - If patient has capacity, document the patient does not consent to specific information (such as third party information) to be disclosed in the clinical notes
 - Document clinically relevant observations or presentations in the notes to ensure appropriate treatment options are still available and offered to the patient
 - A Datix Clinical Incident Management System (CIMS) must be completed within 24 hours of the incident
 - Combined Hazard or Incident Report (CHoIR) form must be completed at the earliest opportunity.

Additionally, for **MH patients** - all reports of alleged sexual safety incidents, alleged sexual assaults and/or alleged sexual contact including retracted disclosures must be reported to the Office of the Chief Psychiatrist, in compliance with the [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#) via [Reporting Notifiable Incidents – Public Mental Health Services](#). CIMS is to be used for this purpose. It is expected that the reviewing Consultant Psychiatrist will, at minimum, provide 3rd party comment or complete the CIMS investigation.

- Reporting to the police
 - Mandatory reporting must be complied with for children and young people under 18 years through [Mandatory Reporting Information System](#) as per **CAHS Guidelines for Protecting Children**
 - Also refer to [4. Establish a brief history of what has happened and when to determine an appropriate course of action](#) section.

Response process – 10. Notifications, mandatory reporting, and documentation cont'd

- Documentation
 - Record in the integrated notes or outpatient notes that an allegation of a sexual safety incident has been reported, including when retracted
 - Ensure comprehensive and contemporaneous documentation pertaining to an alleged or reported sexual safety incident is to be recorded using the patient's words in the:
 - Patient medical record, or
 - If a complaint about service provided by RPBG, in a confidential file separate from the patient's medical record – as per the [WA Department of Health Complaints Management Policy](#) with inclusion of the following:
 - Patient consent regarding reporting to the police
 - Advising family, guardian and/or significant others of treatment
 - Follow up care, referrals and/or other actions taken and/or escalated as necessary
 - Provide the confidential file to the immediate line manager.
 - Add a risk flag to iCM for inpatients and/or place an alert on PSOLIS/ WebPSOLIS for MH patients following known and/or reported incidents within 24 hours of the incident. Refer to RPBG Clinical Handover Policy to communicate risk alerts.

11. Debrief

- Staff are to be aware of the impact of witnessing sexually disinhibited behaviour or a sexual safety incident and provide support to the person witnessing the event whether they are a patient, significant other or visitor
- Line Managers will ensure immediate support and offer or provide debriefing for the staff. Refer to [EMHS Critical Incident Stress Debriefing Policy](#) and [EMHS Employee Assistance Program Policy](#).
- Staff are to encourage the patient and relevant others (non-staff, e.g., volunteers) to receive debriefing and psychological support post incident due to the risk of adverse psychological trauma. Refer to [Mental Health Consumer, Carer, Visitor Support Post-Critical Incident SOP](#).

12. Investigation leads

- The following designation will lead the investigation:
 - General health – Lead Clinician or Consultant
 - Mental health – Clinical Lead
- Report progress and status to:
 - Divisional Directors, Co-Directors, Medical Co-Directors or On Call Executives as appropriate.

Other scenarios to consider

Concerns about the plausibility of a patient's mental state

- Do not make judgements or assumptions based on mental state. In circumstances where a patient alleges a sexual safety incident and staff considers the patient as experiencing acute mental state (e.g., delusional), staff are to document this in the integrated notes and request MO or DMO to complete a medical review and this must be **confirmed** by Treating team or Consultant

Retracted or suspected disclosures

- Record **all** sexual safety breaches, i.e., retracted disclosures, where a breach of sexual safety is suspected but not disclosed, reports of an alleged sexual safety incident and identify priority actions to increase sexual safety of the patient and others even where there is no clear factual basis for the allegation³
- Staff are to discuss with the patient what they have observed and why they are concerned and escalate as necessary.

Where a sexual safety incident is reported by a third party

- Staff are to discuss with the patient what has been reported and why they are concerned and escalate as necessary

Allegations against a RPBG staff member

- Contact SARC
- Notify staff member's line manager or if after hours OHC, AH Duty MH CNS or SNoC who will notify the site Executive and Human Resources Director and Integrity and Ethics in order that the relevant notifications can be made
 - Fitness for work assessment to be considered
- Refer to [EMHS Inappropriate Workplace Behaviours, Reporting of Guideline](#), [WA Department of Health Notifiable and Reportable Conduct Policy](#) and [WA Department of Health Discipline Policy](#) for further information and resources
- Any medical care required is to be accessed from a service other than where the alleged incident occurred and/or encourage GP or ED attendance as applicable
- Provide and/or facilitate access to psychosocial support and information regarding processes and rights to staff member.

Compliance monitoring

Compliance will be monitored as part of the Datix CIMS process and/or CHoIR, OCP Reporting Notifiable Incidents, RPBG Performance Monitoring Schedule and via the Compliments or Complaints Management System and Human Resources disciplinary reporting.

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We acknowledge the previous site endorsed work and/or contributors used to compile this document including:

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Feedback received

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Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Sexual Safety: Responding to Incidents Policy through consultation undertaken through the Aboriginal Health Strategy Team and the Aboriginal Workforce Engagement Group including the Aboriginal Health Liaison Officers, Aboriginal Health Strategy Team, and Aboriginal Acute Care Coordination Team. to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD ID 3005).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

References

1. Department of Health. *Procedure for Responding to a Recent Sexual Assault*. 2017. Available via Sexual Assault Resource Centre . Available from URL: [King Edward Memorial Hospital - Sexual Assault Resource Centre \(SARC\) information and resources for health professionals](#)
2. *Australian Human Rights Commission*. [cited May 2021]. Available from URL: [Sexual Harassment \(humanrights.gov.au\)](#)
3. *Chief Psychiatrists Guidelines for the Sexual Safety of Consumers of MH Services in WA*. 2020. [Cited May 2021]. Available from URL: <https://www.chiefpsychiatrist.wa.gov.au/standards-guidelines/sexual-safety-guidelines/>

Related policy, practice standard, clinical guidelines

Not already listed

RPMG Policy Hub

- [Community Mental Health Service Triage to Discharge SOP](#)
- [Community Mental Health Home Visiting SOP](#)
- [General Health Admission, Transfer and Discharge SOP](#)
- [Escalation to Consultant Policy](#)
- [Visitor Management Policy](#)

EMHS Policy Hub

- [Acute Response Mental Health Emergencies/Crisis Policy](#)
- [Community/Home Visiting Policy](#)
- [Notifications requirements under the WA Mental Health Act 2014 Policy](#)
- [Consent Policy](#)
- [Mental Health Advocates Policy](#)

WA Department of Health Policy Hub

- [Procedure for Responding to a Recent Sexual Assault](#)
- [Responding to a Recent Sexual Assault: Supporting Information](#)
- [Statewide Standardised Clinical Documentation \(SSCD\) Policy](#)
- [Safety Planning for Mental Health Consumers Policy](#)
- [Code of Conduct Policy](#)

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Authorised by: No Patient Harm Committee 28/05/2024

Date Compiled:

03/2018

Document owner:

Director of Clinical Services

Revision Due:

05/2027

Related policy, practice standard, clinical guidelines cont'd

Other

- [SARC](#)
- [Chief Psychiatrist's Sexual Safety Guidelines](#)
- [The Australian Open Disclosure Framework – Australian Commission on Safety and Quality in Health Care \(ACSQHC\)](#)

Legislation

- [Mental Health Act 2014](#)
- [Criminal Code Act Compilation 1913](#)
- [Criminal Law Amendment \(Sexual Assault and Other Matters\) Act 2004](#)
- [Children and Community Services Act 2004](#)
- [Commonwealth Aged Care Act 1997](#)

Links to resources

- [Non-Fatal Strangulation \(NFS\) – SARC WNHS](#)

Legislative requirements, the evidence, and the standard

For information relating to the legislative requirements and standards that RPBG policy documents must adhere to, and regarding the logos and levels of evidence used within RPBG policy documents, refer to [Legislative Requirements, the Evidence and the Standard](#) (live link) on the Policy Hub.

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021)

Standard 1: Clinical Governance

Standard 5: Comprehensive Care

Standard 6: Communicating for Safety

NSMHS Standards (2010)

Standard 1: Rights and Responsibilities

Standard 2: Safety

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